February 23, 2026

Thomas Keane, MD
Assistant Secretary of Health and Human Services
U.S. Department of Health & Human Services
200 Independence Ave. SW
Washington, D.C. 20201

Submitted Electronically

Dear Assistant Secretary Keane:

Orion Health appreciates this opportunity to respond to the *Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care*.

Orion Health, a HEALWELL AI company, is a global healthcare technology firm specializing in interoperability, health data infrastructure, and population-level data integration. In the United States, Orion Health partners with state governments, health systems, health information exchanges, clinically integrated networks, and risk-bearing entities to enable secure data exchange, improve care coordination, and support value-based care initiatives.

Orion Health's 30+ years of experience operating longitudinal exchange networks and integration platforms positions us to contribute practical implementation insight as these policies continue to evolve.

We welcome continued dialogue and collaboration to help translate these strategic priorities into operational reality.

Yours sincerely,

/s/

James Henderson
SVP and General Manager, U.S.

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From Friction to Function: Interoperability as the Foundation for Scalable Clinical AI

Submitted in Response to RIN 0955-AA13
Orion Health

Perspective from Operating Longitudinal Health Data Infrastructure

Recent federal discussions, including the ASTP Annual Meeting, highlight a pivotal shift: U.S. healthcare is constrained more by coordination than by digitization. Data exists across systems, yet often lacks liquidity. When information does not move reliably across settings, it creates operational friction for clinicians, patients, and families.

Across the regional and national data exchange environments, payer and provider integration platforms, and longitudinal patient record systems we support, we have observed this dynamic firsthand. In our experience, the durability of clinical AI depends fundamentally on the integrity, completeness, and governance of the underlying data environment.

AI adoption is underway. The critical question is whether it will scale within an infrastructure that supports trust, transparency, and sustained performance.

OneHHS Alignment: Coordination Enables Innovation

The OneHHS approach, emphasizing coordination across ASTP, CMS, FDA, NIH, and other agencies, reflects an important reality. Interoperability policy, reimbursement reform, regulatory oversight, and research investment cannot evolve independently.

In practice, fragmentation in policy can mirror fragmentation in data. Cross-agency alignment creates the predictability necessary for responsible innovation. When interoperability standards, payment models, and AI governance frameworks move in concert, the ecosystem can scale proven solutions with confidence.

We view regulatory clarity not as a constraint on innovation, but as an enabler. Clear expectations reduce hesitation and accelerate adoption.

Longitudinal Interoperability as a Precondition for Reliable AI

In multi-state and multi-organizational settings where Orion Health aggregates records across hospitals, ambulatory providers, laboratories, imaging systems, and payers, longitudinal completeness materially improves analytic reliability.

We have seen this particularly in:

- Chronic disease management programs requiring a multi-year history
- Cross-setting medication reconciliation
- Emergency-to-primary care coordination
- Population risk identification incorporating social and behavioral context

AI models developed within narrow institutional boundaries, often show performance variability when patients receive care elsewhere. In contrast, models evaluated within governed, interoperable environments are more resilient and transparent.

As TEFCA adoption accelerates and exchange volume expands, interoperability is transitioning from policy aspiration to operational infrastructure. The strategic focus appropriately shifts from simple connectivity to performance, normalization, governance, and activation within clinical workflow.

From our perspective, AI and interoperability are layered components of the same modernization strategy, rather than parallel efforts.

Risk-Based Oversight and Lifecycle Governance

Federal leadership emphasizes proportionate, risk-based oversight that reduces unnecessary compliance burden while maintaining safeguards where clinical impact is highest.

In live deployments, we see that AI initiatives progress most effectively when supported by infrastructure that enables:

- Transparent documentation of data provenance
- Clear articulation of intended use and limitations
- Multi-site validation across diverse populations
- Ongoing lifecycle monitoring for drift and bias

Beyond technical infrastructure, durable AI adoption also depends on disciplined evaluation and scaling practices, including:

- Quantitative measurement of clinical, operational, and cost impact to guide decisions
- Explicit criteria to distinguish exploratory pilots from enterprise deployment

Infrastructure supporting traceability and auditability lowers barriers to responsible innovation. Predictable, risk-calibrated oversight enables better alignment among stakeholders.

Innovation and safety need not be opposing forces. They are mutually reinforcing when supported by mature data environments.

Real-Time Integration and Administrative Modernization

Federal initiatives such as electronic prior authorization, real-time prescription benefit integration, and provider directory modernization reflect practical friction points encountered daily in healthcare delivery.

In payer and provider integrations we support, reliable endpoint routing, accurate directory information, and consistent API performance are essential for automated transactions functioning at scale. Minor inaccuracies can disrupt high-volume workflows and undermine trust.

Similarly, in administrative automation pilots, standardized transaction frameworks reduce manual burden only when exchange pathways are predictable and structured data is consistently available.

These operational realities reinforce the need for enforceable interoperability. The goal is not merely the ability to connect, but reliable, performant, and transparent connectivity.

AI embedded in real-time integration pathways can reduce documentation burden, identify risk earlier, and assist clinical decision-making—if it operates within trusted infrastructure.

Prevention, Healthspan, and Data Liquidity

Federal dialogue is shifting from reactive intervention to proactive management, improving healthspan, addressing chronic disease, and integrating behavioral and social context.

Such objectives depend on longitudinal visibility across time and care settings. In environments where clinical, preventive, and social data are harmonized into unified records, analytic capabilities expand significantly. Predictive insights become more equitable, and interventions can be targeted more effectively.

Infrastructure that enables patients and clinicians to access comprehensive records and supports appropriate consented sharing, strengthens both care coordination and AI-driven intelligence.

Guiding Principal for AI adoption

Based on our operational experience, a constructive guiding principle for clinical AI adoption is:

AI that meaningfully influences patient care is most durable when grounded in interoperable, longitudinal data environments and validated through transparent, evidence-informed processes.

Federal efforts to align exchange frameworks, API performance standards, evolving value-based reimbursement, and risk-calibrated AI governance signal a coordinated modernization strategy. The convergence of these initiatives, rather than any single rule, will determine whether AI scales responsibly.

In practice, infrastructure enables liquidity. Liquidity enables reliable intelligence. Reliable intelligence, responsibly governed, supports better care.

The direction articulated across ASTP, CMS, FDA, and collaborating federal partners reflects a shared objective. The goal is to reduce friction, improve coordination, strengthen affordability, and accelerate responsible innovation.